Venkata Lakshmi Parimala Pasupuleti

Applied AI Engineer · Python / LangGraph / LLMs · HITL & Eval Infrastructure · HIPAA / Healthcare AI

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Date: September 23, 2026
To: Hiring Team, Parakeet Health
Re: Software Engineer, AI Products

I built LeaseI with one design rule that I never bent: the LLM extracts and reasons; a human makes every final regulated call. Tenant eligibility decisions - rent-to-income thresholds, lease compliance flags, housing voucher determinations - never resolve autonomously. The agent surfaces structured evidence, scores confidence, and hands off. I read your job description and recognized that rule immediately. You're building the same boundary for patient data.

LeaseI is an 8-agent LangGraph system with RAG retrieval, OCR ingestion, structured data extraction, and HITL guardrails on regulated decisions - evaluated across 1,240 test cases at 93.6% precision and 87.9% F1. The fax processing product you've built (50,000 faxes/month → automated scheduling in under a minute) uses exactly the same architecture: ingest unstructured documents, extract structured fields, route edge cases to a human. I've shipped that in the lease compliance domain; the patterns transfer directly.

For eval infrastructure: I built AgentProbe specifically to answer the question "is this agent ready for production before it touches real users?" - CI-integrated regression gate, LLM-as-judge scoring, hallucination detection, pass@k harness, agent tracing, multi-provider LLM routing. Your description calls out building evaluation systems "that can tell the difference between a good fix and a confident wrong answer." That is the exact problem AgentProbe is designed to solve.

On HIPAA: I have two independent proof points. AI on FHIR is a production-deployed HIPAA-compliant AI system I built over structured PHI - RBAC, NLP-to-FHIR API translation, LLM and BERT integration. At Entrata, I owned AWS S3 migration with Libsodium-encrypted pipelines across 7 years - 100% PHI compliance, zero security incidents. Healthcare AI requires both the technical capability and the instinct for what data can and cannot flow where. I've built both.

I want to address Django directly: it's not in my stack. Flask and FastAPI are. For an experienced Python engineer, Django is days of ramp, not months - but I'd rather say that plainly than paper over it. What I do bring is 9 years of production Python, two HITL/eval systems the team can walk through, and the judgment that comes from owning healthcare-regulated data at scale. I'm ready to ship Day 1 on the problems that matter.

🦜 I'm excited about Parakeet Health specifically because the engineering problems are real and the patient impact is concrete. I'd love to talk.

Sincerely,

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